

# Urinary Tract Infections (UTIs)

Patient Information · What a UTI is, how it is treated, and how to prevent it

WARWIKI

A urinary tract infection (UTI) happens when germs called bacteria grow in the urinary tract. Most UTIs are **bladder** infections. They cause burning and a need to urinate often. Less often the infection reaches a **kidney**, which is more serious. Most UTIs clear quickly with the right antibiotic, and there are good ways to prevent repeat infections.

## About This Condition

A bladder infection (**cystitis**) usually causes:

- **Burning or stinging** when you urinate
- **A strong urge to go**, and going often in small amounts
- Discomfort low in the belly

A **kidney infection** (pyelonephritis) is more serious. It adds **fever, chills, pain in the side or back, and nausea**. It needs prompt care.

## What Causes It

Bacteria, usually from the bowel, enter through the urethra (the tube you urinate from) and grow in the bladder. Some things raise the risk:

- **Sex**, and use of spermicide
- **Menopause**, because of lower estrogen
- Not fully emptying the bladder, or a bladder or vaginal bulge (prolapse)
- Catheters, stones and diabetes

## Learn the Terms

### UTI

An infection anywhere in the urinary tract.

### Cystitis

A bladder infection. This is the most common UTI.

### Pyelonephritis

A kidney infection. It is more serious and often causes fever.

### Urine culture

A test that grows the bacteria to help pick the right antibiotic.

### Recurrent UTI

Repeat infections: 2 in 6 months, or 3 in a year.

### Vaginal estrogen

A low-dose cream, tablet or ring used in the vagina. It lowers UTI risk around and after menopause.

### Methenamine

A daily pill that helps prevent UTIs. It is not an antibiotic.

**Do I always need antibiotics?** If you have symptoms, a UTI is treated with antibiotics. But if a urine test finds bacteria and you feel **fine**, it is usually **not** treated. In that case, antibiotics do more harm than good. The main exceptions are pregnancy and some urinary procedures.

## How It's Diagnosed and Treated

- Your team looks at your **symptoms** and a **urine test**. Give a urine sample **before** you start antibiotics, if you can.
- A bladder infection is treated with a **short antibiotic course**.
- A **kidney infection** may need a longer or stronger antibiotic, and sometimes hospital care.
- Take every dose, even once you feel better.

## Preventing Repeat UTIs

- If you drink little, **drinking more water** can help.
- Avoid spermicide, a gel used for birth control.
- Around and after menopause, **vaginal estrogen** lowers the risk. After menopause, it cuts UTIs by about half.
- **Cranberry** products may lower the number of UTIs by about a quarter.
- **Methenamine** is a non-antibiotic pill that can help.
- If these are not enough, ask about a low-dose antibiotic taken daily or after sex.

## Good to Know

- **D-mannose** alone has not been shown to prevent UTIs.
- Urinating after sex and wiping front to back are fine to keep doing, but they are not proven to prevent UTIs.
- Repeat UTIs may lead to a check of how well you empty your bladder.

### Call the office that is treating you if you have:

- A temperature of **100.4 °F (38 °C)** or higher, chills, or pain in your side or back
- Blood in the urine, or symptoms that are not getting better on antibiotics
- Any UTI symptoms while pregnant

### Go to an emergency department if:

- You have fever and keep vomiting, or cannot keep pills down
- You feel faint, confused or very unwell
- You cannot reach the office and are worried

## Three Things to Remember

1. Most UTIs are bladder infections and clear with a short antibiotic course.
2. Bacteria in the urine **without symptoms** usually should not be treated.
3. Vaginal estrogen around and after menopause, cranberry and methenamine can help prevent repeats.